EVIDENCE BRIEF · SAMPLE

Long COVID: what the research says about death, heart events and time off work

For: life, health, critical illness and disability insurance teams · Date: 6 Nov 2026 · Basis: paper abstracts only (see page 2)

Bottom line. Long COVID is a morbidity story more than a death story. In a large US veterans study, people who were not hospitalised had no extra risk of death after the first year. People who were hospitalised still had about 29% higher death rates in year three. For disability and sick-pay claims the picture is clearer: in a Saudi cohort only 39% of long COVID patients were back at work normally within 12 months, against 82% of those who recovered. About half of people with long COVID at 12 months improved meaningfully by 36 months, and about a quarter recovered fully.

1. What the evidence shows

QUESTION	FINDING	SOURCE	STRENGTH
Death after infection, not hospitalised	Extra risk of death "no longer present after the first year"	US Veterans Affairs cohort: 135,161 infected, 5.2 million controls, 3 years (Cai 2024)	Strong on size; mostly older male veterans
Death after infection, hospitalised	Still raised in year 3: rate ratio 1.29 (95% CI 1.19 to 1.40)	Same	Strong on size
Death within 1 year of hospital discharge	15% (382 of 2,535 patients); a quarter had persistent symptoms at 9 to 12 months. 64% of patients were from low- and middle-income countries	16-country study (Sliwa 2025)	Moderate: hospitalised patients only, no uninfected comparison
Heart events and death, long COVID vs infected without it	Heart events: hazard ratio 4.48 (3.95 to 5.07). Death: 1.53 (1.38 to 1.69)	Hospital records, 2020 to 2023 (Tsai 2026)	Weak to moderate: records-based, sicker patients get the label
Recovery	Of people with likely long COVID at 12 months: 36.2% improved by 24 months, 50.1% by 36 months; 26.9% reached no symptoms by 36 months	US cohort, 1,019 people (Reeder 2026)	Moderate: observational
Back at work within 12 months	38.7% with long COVID vs 82.3% who recovered	Saudi cohort, 816 people, median 24 months (Kamal 2025)	Moderate: single region
Who is more likely to get it	Female sex and diabetes (very wide ranges); delayed recovery with reinfection and hospitalisation	Same Saudi cohort	Weak to moderate: wide confidence ranges

2. How to read the numbers

- **The 4.48 heart-event figure is probably too high.** People who get a long COVID diagnosis are often already sicker, and the study compares them with infected people who were not diagnosed. In my judgment (about 70%) a good part of that gap is not caused by long COVID. The 1.53 death figure has the same problem.
- **Different groups, different answers.** The veterans cohort is mostly older men in the US. The Saudi and 16-country cohorts are closer to Gulf and South Asian populations but have no uninfected comparison group, so they say less about excess risk.
- **Most studies follow people for up to 3 to 5 years.** Nothing here covers 10 years or more.

3. What this could mean for a claims or product team (my judgment, not a finding)

- **Disability and critical illness:** the work-return gap is the strongest signal. A claim with a long COVID diagnosis and ongoing symptoms at 12 months is likely to run longer than a typical short-term claim (about 65% in my judgment), but about half of such claimants improve within three years, so it should be reviewed at set intervals, not assumed permanent.
- **Life underwriting:** little reason from this evidence to load premiums for people who had COVID and were not hospitalised. A hospitalised case may deserve a look in the first 3 years.
- **Questions to ask:** was the person hospitalised, how long have symptoms lasted, any diabetes, reinfections, and a doctor's record of work capacity.

4. Sources (with identifiers)

1. Cai M et al. Nat Med 2024. DOI 10.1038/s41591-024-02987-8 · PMID 38816608
2. Tsai Y-T et al. BMC Cardiovasc Disord 2026. DOI 10.1186/s12872-026-06026-x · PMID 42178526
3. Reeder HT et al. Ann Intern Med 2026. DOI 10.7326/ANNALS-25-05275 · PMID 42970801
4. Sliwa K et al. Glob Heart 2025. DOI 10.5334/gh.1452 · PMID 40757253
5. Kamal SM et al. BMC Infect Dis 2025. DOI 10.1186/s12879-025-11468-3 · PMID 40883696

5. What this brief is not

An evidence summary for discussion. It is not medical advice, an underwriting guideline, an actuarial assumption or legal advice. It was built from abstracts found through the Firecrawl research index and PubMed, not from full papers. Numbers are quoted as the abstracts give them, not re-computed. Sections 1 and 4 report the papers; sections 2 and 3 are my judgment. Prepared by Syed Shafai, syed.shafai@gmail.com. Abstract data from PubMed.